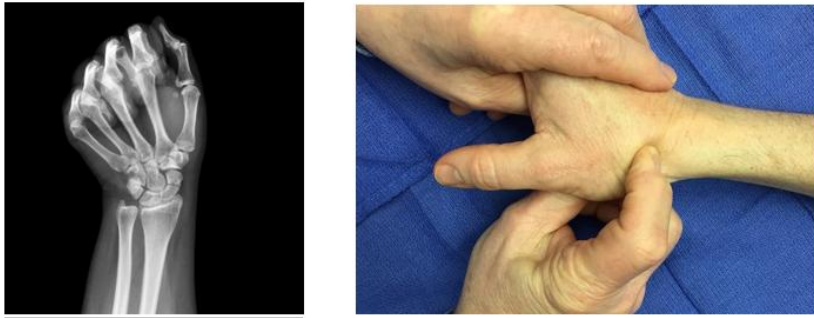




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QUESTIONS

Q1. A young male patient fell on while playing and started complaining of Acute pain in the wrist region. His examination revealed:



Most common complication of this fracture in clinical practice is :

- a) Malunion
- b) Avascular necrosis
- c) Non union
- d) Wrist Osteoarthritis

Q2. Which of the following injuries is classified as Gustillo – Anderson Grade III injury ?

- a) Open fracture with clean wounds less than 1 cm long
- b) Open fractures with laceration more than 1 cm long usually upto 10 cm, with extensive tissue damage , flaps or avulsions
- c) Open segmental fractures with extensive soft tissue damage
- d) Open fracture with Compartment syndrome

Q3. What's the diagnosis of X ray picture shown below ?



- a) Essex-Lopresseti Fracture Dislocation

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- b) Galeazzi Fracture Dislocation
- c) Monteggia Fracture Dislocation
- d) Bennet's Fracture Dislocation

Q4. Terrible triad of elbow is known as:

- a) Humerus fracture with medial and Lateral epicondyle fracture
- b) Shaft fracture with dislocation and ulnar fracture
- c) Elbow dislocation with Radial and Ulna shaft fracture
- d) Elbow dislocation with radial head and coronoid fracture

Q5. 20 year old boy presented to the emergency with complaints of Pain around Lateral aspect of Wrist after hitting with a closed fist. X Rays taken for the patient are shown below.



The fracture pattern described is difficult to maintain in a reduced position mainly because of pull of :

- a) Flexor Pollicis Longus
- b) Abductor Pollicis Longus
- c) Adductor Pollicis
- d) Extensor Pollicis Longus

Q6. Following anterior dislocation of the shoulder, a patient develops weakness of flexion at elbow and lack of sensation over the lateral aspect forearm. nerve injured is:

- a) Radial nerve
- b) Musculocutaneous nerve
- c) Axillary nerve
- d) Ulnar nerve



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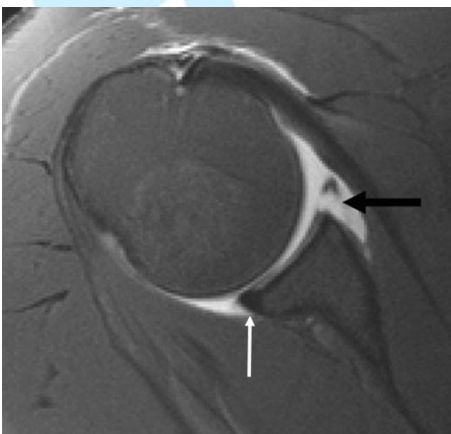
- Q7. A 7yrs old child fell down while playing and presented with c/o pain and swelling around the elbow. He had significant restriction of movements at the Elbow and X Rays showed:



All the following statements regarding the abovementioned fracture are true except:

- a) Clinical examination reveals preserved 3 point bony relationship around elbow
- b) Progressively increasing Cubitus Varus is M/C fracture complication
- c) It is the m/c fracture associated with vascular Injury
- d) M/C type is Extension type of fracture

- Q8. A sportsman presented with complaints of Pain and instability in his shoulder joint since last 3 years. Careful examination revealed multiple dislocations of the same joint . MRI taken revealed:



What should be next step in management:

- a) Arm Sling with Analgesics
- b) Physical therapy with strengthening exercises
- c) Surgical repair
- d) Hanging Cast followed by Activity modification

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Q9. A 7yrs old child sustained an elbow injury due to fall on an outstretched arm with varus stress at the elbow. X rays taken at the hospital showed:



Ideal management of the fracture is :

- a) Excision of the fragment
- b) Open Reduction & Internal fixation
- c) Closed Reduction & Internal fixation
- d) Closed Reduction and Above Elbow POP Cast

Q10. During evaluation a dye was injected in one of the extremities in a child. It was followed by pain and swelling of the upper limb, paresthesias of fingers, stretch pain and normal peripheral pulses. The management of the condition is :

- a) Aspiration
- b) Anti- Inflammatory medications and sling
- c) Observation
- d) Urgent fasciotomy

Q11. Following X Ray shows which type of Epiphyseal Injury pattern :



- a) Type 1

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- b) Type 2
- c) Type 3
- d) Type 4

Q12. A 3 yrs old child holding her father's hand slipped and fell but did not let go off her father's hand. After that she continued to cry due to which she was brought at the hospital by her father. Orthopaedic examination revealed restricted elbow movements but no obvious crepitus or any abnormal mobility. She was afebrile, x rays unremarkable and blood tests were within normal limits. Most appropriate next step would be :

- a) Examination under General anaesthesia
- b) Supinate the forearm & flex the elbow
- c) Limb elevation and Above Elbow POP Cast
- d) Further testing to r/o osteomyelitis

Q13. A 65 yrs old patient tripped and fell down. After the injury, patient complained of pain and swelling around wrist region. Thorough evaluation was done and a POP cast was applied for fracture management as shown below:



All are true of the fracture except :

- a) Most of these fractures are managed non operatively with POP Cast
- b) Finger stiffness is most common complication
- c) Cast is applied with wrist in a position of Palmar flexion
- d) Cast is applied with wrist in a position of Dorsiflexion.

Q14. A 40 yrs old Female presented to the clinic after 3 months of traumatic foot injury with H/O pain and swelling of the leg and foot since 8-10 days. Her skin on examination was cold, shiny and edematous. There was no H/o DM or hypertension.



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The most probable diagnosis is :

- a) Fibromyalgia
- b) Complex Regional Pain Syndrome type I
- c) Peripheral Neuropathy
- d) Osteomyelitis

Q15. All of the following are true regarding the Whiplash injury except :

- a) Cervical Spine is Most commonly involved
- b) It is a Hyperextension injury
- c) It is a Hyperflexion Injury
- d) Sprains and strains without any obvious fracture

Q16. A 45 yrs old female presented with c/o pain in Right Hip along with inability to walk following a fall while walking. O/E there was tenderness in Scarpa's triangle. X Rays were normal. The next best investigation is :

- a) CT Scan
- b) MRI Scan
- c) Bone Scan
- d) Aspiration from joint.

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Q17. A young soccer player sustained an injury while playing and presented to the Orthopaedic surgeon with c/o Pain and Instability in the knee joint. The clinical test done below is for identification of which injury pattern?



- a) Medial Collateral Ligament
- b) Lateral Collateral Ligament
- c) Anterior Cruciate Ligament
- d) Posterior Cruciate Ligament

Q18. A 30 yrs male patient sustained injury to the Left Hip and was brought in the casualty. He was unable to bear weight on the affected limb. The x Rays taken showed:



His Left Lower limb will show :

- a) Abduction, External Rotation & Lengthening of limb
- b) Abduction, External Rotation & Shortening of limb
- c) Adduction, Internal Rotation & Lengthening of limb
- d) Adduction, Internal Rotation & Shortening of limb



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Q19. Identify the splint :



- a) Russel's Splint
- b) Thomas Splint
- c) Crammer Wire Splint
- d) Bohler Braun Splint

Q20. A 40 years old man, was admitted with fracture shaft femur following a road traffic accident. On 2nd day he became disoriented. He was found to be tachypnoeic, and had conjunctival petechiae. Most likely diagnosis is:

- a) Pulmonary embolism
- b) Sepsis syndrome
- c) Fat embolism
- d) Haemothorax

Q21. An elderly lady with Severe Inflammatory Arthritis and on long term medications for treatment of Osteoporosis suddenly presented in emergency with pain in thigh with Inability to ambulate. X Rays revealed:



What is the most probable diagnosis ?



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- a) Atypical Fracture
- b) Typical Subtrochanteric Fracture
- c) Paget's Disease of Bone
- d) Metastasis

Q22. A young male patient was hit by a vehicle while walking on road. He fell down and was immediately brought to the ER of the hospital. He was unable to stand up and bear weight on the affected limb & significant knee swelling developed. X Rays taken revealed:



All the following can be seen on examination of the injured limb except:

- a) Weakness in Ankle Dorsiflexion
- b) Weakness in Toe extension
- c) Weakness in Foot Eversion
- d) Weakness in Ankle Plantarflexion

Q23. 82 yrs old female fell at her home and presented at the hospital with complaints of Pain in hip region. On Examination , the injured limb was externally rotated , active SLR was painful and limited & distal neurovascular status was intact . X Rays done revealed:



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What is the best treatment option for her ?

- a) Proximal femoral nailing
- b) Hemiarthroplasty
- c) Internal Fixation with Screws
- d) Total Hip Replacement

Q24. All of the following statements regarding Meniscus are true except :

- a) Medial Meniscus is more mobile than Lateral meniscus
- b) Lateral Meniscus covers more tibial articular surface than Medial Meniscus
- c) Medial Meniscus is more commonly injured than the Lateral Meniscus
- d) Menisci are predominantly made up of Type 1 collagen.

Q25. Investigation of choice for below mentioned injury pattern is :



- a) X Ray



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- b) MRI Scan
- c) CT Scan
- d) Ultrasound

Q26. A 48-year-old woman presents with diffuse bone pain, difficulty climbing stairs, and progressive proximal muscle weakness for 1 year. She has a waddling gait. X-ray shows transverse lucent bands perpendicular to the cortex in the femoral neck region. Laboratory findings are :

Serum calcium: low-normal

Serum phosphate: low

Serum Vitamin D : Low

ALP: markedly elevated

What is the most likely diagnosis?

- a) Osteoporosis
- b) Osteomalacia
- c) Hyperparathyroidism
- d) Paget disease

Q27. A 72-year-old man complains of progressive enlargement of skull size and decreased hearing. Examination reveals bowing of tibia. Serum alkaline phosphatase is markedly elevated with normal calcium and phosphate. Six months later he develops severe localized pain and swelling over femur. Skull X rays taken show:



Most likely complication ?

- a) Osteomyelitis
- b) Giant cell tumor
- c) Osteosarcoma



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d) Multiple myeloma

Q28. A 5-year-old child presents with recurrent fractures and progressive visual impairment. Examination reveals hepatosplenomegaly and anemia. X-ray shows diffusely sclerotic bones with “bone within bone” appearance. What is the primary pathological defect?

- a) Failure of osteoblast mineralization
- b) Excess osteoid production
- c) Failure of osteoclast-mediated bone resorption
- d) Defective type I collagen synthesis

Q29. All of the following are true about Radiological features of Rickets , except :

- a) Narrowing of Physis
- b) Cupping and Splaying of Metaphysis
- c) Delayed appearance of Epiphysis
- d) Generalized Osteopenia

Q30. All are components of Frax Scoring except :

- a) H/O Glucocorticoid usage
- b) H/O Alcohol intake < 3 units/day
- c) H/O Rheumatoid arthritis
- d) H/O Secondary Osteoporosis

Q31. A patient develops weakness in extension of fingers after proximal forearm fracture. On further examination, his wrist extension is preserved but there is deviation radially. Patient does not have any sensory deficit in the affected limb. Most likely spared muscle?

- a) Extensor digitorum Communis
- b) Extensor indicis
- c) Extensor carpi radialis longus
- d) Extensor pollicis longus



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Q32. A patient with chronic cubital tunnel syndrome develops weakness of interossei and lumbricals. Surprisingly, clawing is less severe than expected. This phenomenon occurs because:

- a) FDP to ring and little finger is denervated
- b) Median nerve compensation occurs
- c) Extensor expansion remains intact
- d) Adductor pollicis becomes hyperactive

Q33. All of the following statements are true regarding Brachial Plexus Injury , except :

- a) Erb's Palsy is an injury to Upper Trunk and has the best prognosis amongst Brachial Plexus Injuries
- b) Klumpke's Palsy is injury to Lower Trunk and may be associated with Horner's Syndrome
- c) Preganglionic lesions have better prognosis than Post Ganglionic Lesions
- d) Histamine Test is used to differentiate between Preganglionic and Postganglionic Lesions

Q34. In the clinical test shown , The Right Hand show's weakness of which muscle and which peripheral nerve palsy is the cause for it ?



- a) Abductor Pollicis Brevis; Median Nerve
- b) Adductor Pollicis ; Ulnar Nerve
- c) Adductor Pollicis ; Median Nerve
- d) Abductor Pollicis Brevis ; Ulnar Nerve

Q35. A middle aged gentleman presented to Orthopaedic OPD with complaints of Pain and Paresthesia over the anterolateral aspect of thigh. On examination his motor power and Deep tendon reflexes in lower limbs were normal. What is the most probable diagnosis :

- a) Peripheral Vascular Disease
- b) Entrapment of Femoral Nerve under Inguinal Ligament

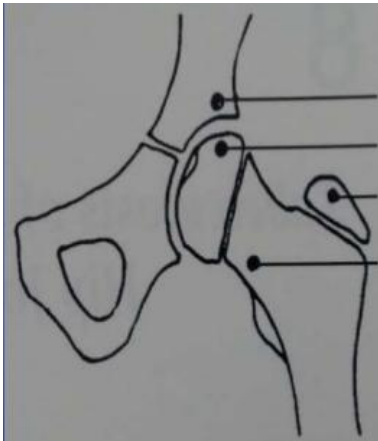


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- c) Paracentral Disc Prolapse L4/5
- d) Entrapment of Lateral Cutaneous Nerve of Thigh

Q36. Initial Focus of TB Hip is :



- a) Greater Trochanteric Region
- b) Acetabular Roof
- c) Femoral Head
- d) Femoral Neck

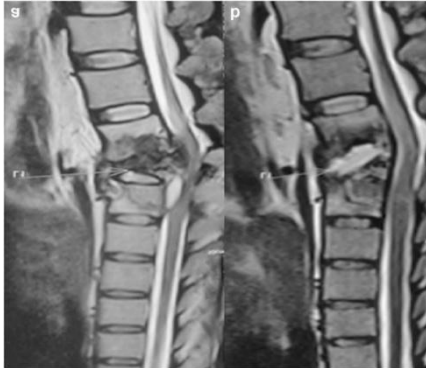
Q37. An 8-year-old boy presents with fever, severe pain around the knee, and refusal to bear weight for 5 days. Examination reveals tenderness over proximal tibia with local warmth. ESR and CRP are elevated. Most common organism responsible?

- a) Salmonella
- b) Pseudomonas
- c) Staphylococcus aureus
- d) Mycobacterium tuberculosis

Q38. A 35yrs old male patient presented with Back pain for 3 months along with decreased spinal movements on examination. He also had associated loss of weight and a significant decrease in his appetite. MRI done revealed:



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The most common radiological presentation of the above pathology is :

- a) Anterior Subligamentous
- b) Posterior / Neural Arch
- c) Paradiscal
- d) Central

Q39. A 6-year-old child presents with High-grade fever and Severe hip pain along with refusal to bear weight on the affected limb. On examination, Hip was held in flexion, abduction, and external rotation. ESR and CRP are markedly elevated. Most appropriate next step should be :

- a) Skin traction
- b) Oral antibiotics only
- c) Urgent arthrotomy and drainage
- d) Observation for 48 hours

Q40. A 14-year-old boy presents with intermittent fever, dull aching pain in the tibia, and pus discharge from a sinus over the leg for 4 months. X-ray shows a dense fragment of dead bone surrounded by a shell of new bone formation. What is the dense fragment called?

- a) Cloaca
- b) Involucrum
- c) Sequestrum
- d) Brodie abscess

Q41. A 62 yrs old female presented in the hospital with complaints of pain in both her knee joints (R > L) for several years with associated difficulty in walking . Her quadriceps muscles are wasted and clinically she has following appearance:



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All the following are expected findings in her knee x rays except :

- a) Subchondral Sclerosis
- b) Osteophytes and loose bodies
- c) Reduced lateral joint space
- d) Reduced medial joint space

Q42. A 47 yrs old male alcoholic patient presented with complaints of pain in his left knee along with swelling and significant skin redness. There was no significant H/O any trauma. Blood tests revealed elevated ESR and CRP levels. Synovial fluid showed negative birefringence under polarized microscope . What is the most probable diagnosis ?

- a) Septic Arthritis
- b) Gout
- c) Pseudogout
- d) Early Osteoarthritis

Q43. A 45 yrs old female presented in Orthopaedic OPD with c/o Pain & swelling in multiple joints. On careful history taking she c/o stiffness on getting up in morning. The changes of abovementioned clinical pathology are most likely to involve :

- a) Wrist Joint
- b) Metacarpophalangeal (MCP) joint
- c) Proximal Interphalangeal (PIP) joint
- d) Distal Interphalangeal (DIP) joint

Q44. A 26-year-old male with back ache, morning stiffness and reddening of eyes. X- ray of the patient is given below, Identify the diagnosis of this patient:



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- a) Rheumatoid arthritis
- b) Psoriatic arthritis
- c) Ankylosing spondylitis
- d) Paget's disease

Q45. A 42-year-old man presents with chronic knee pain, intermittent locking, and swelling for 2 years. X-ray shows multiple calcified intra-articular loose bodies of similar size. X Ray taken shows:

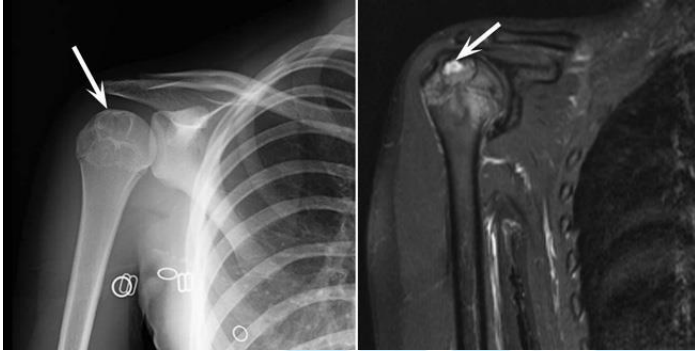


Most likely diagnosis?

- a) Pigmented villonodular synovitis
- b) Osteoarthritis
- c) Synovial chondromatosis
- d) Charcot joint

Q46. A 14 yrs old boy presented with painful swelling over shoulder . Radiograph of the shoulder showed an Osteolytic area with stippled calcification over proximal humerus epiphysis .

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Biopsy of lesion revealed an immature fibrous matrix with scattered giant cells. Which of the following is the most likely diagnosis ?

- a) Giant Cell Tumour
- b) Chondroblastoma
- c) Osteogenic Sarcoma
- d) Chondromyxoid Fibroma

Q47. A 15 yrs old boy is injured while playing. X Rays done rule out a fracture but radiologist reports a lesion with Bony destruction and soft tissue mass. Bone Biopsy reveals a Bony Tumour with neural differentiation . Which is the most likely diagnosis?

- a) Chondroblastoma
- b) Ewing's Sarcoma
- c) Neuroblastoma
- d) Osteosarcoma

Q48. A 17 year old patient presents with a growth behind the heel on talus as shown in the image below. Identify the condition :



- a) Enchondroma



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- b) Osteosarcoma
- c) Osteochondroma
- d) Osteomyelitis

Q49. A 15 yrs old male patient presented with chronic knee pain for 6 months. He has a history of progressively increasing swelling in the lower thigh region. X rays taken show which periosteal reaction ?



- a) Sunray appearance
- b) Soap bubble appearance
- c) Codman's Triangle
- d) Onion peel appearance

Q50. A 70 yrs old Male patient presents with Multiple Bone pains, on evaluation he has a high ESR, high Calcium levels but a normal Alkaline Phosphatase. X Rays reveal pathological vertebral fractures without any H/O significant trauma. Most important investigation to arrive at a diagnosis :

- a) Serum Acid Phosphatase
- b) CT Head with Contrast
- c) Whole Body Scan
- d) Serum Electrophoresis



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ANSWERS

1	b
2	c
3	a
4	d
5	b
6	b
7	b
8	c
9	b
10	d

11	d
12	b
13	d
14	b
15	c
16	b
17	a
18	a
19	d
20	c

21	a
22	d
23	b
24	a
25	b
26	b
27	c
28	c
29	a
30	b

31	c
32	a
33	c
34	b
35	d
36	b
37	c
38	c
39	c
40	c

41	c
42	b
43	b
44	c
45	c
46	b
47	b
48	c
49	c
50	d